

- (a) Non-verbal communication
  - (b) Verbal communication
  - (c) Touch
  - (d) Vision
89. Which of the following solutions has the same osmotic pressure as blood
- (a) Hypertonic solution
  - (b) Isotonic solution
  - (c) Hypotonic solution
  - (d) All of the above
90. \_\_\_\_\_ is a qualitative statement in monetary terms set by an organization for achieving its goals and objectives is known as
- (a) Planning
  - (b) Budgeting
  - (c) Fund raising
  - (d) Insurance
91. Planning can be best described as \_\_\_\_\_
- (a) Designing a means to the end
  - (b) Concrete action to be taken
  - (c) Goals of care
  - (d) The basis for evaluation
92. Members of a group who are equals usually in terms of age, education and social class are referred to as \_\_\_\_\_
- (a) Group tension
  - (b) Dynamic group
  - (c) Secondary group
  - (d) Peer group
93. Which of the following is the best indication that a nasogastric tube in the stomach is properly placed?
- (a) Client is unable to speak
  - (b) Client gags during insertion
  - (c) PH of the aspirants is less than 5
  - (d) Fluid is easily instilled into the tube
94. When a nurse does not act in accordance with professional standards of care, such act is



- (a) Negligence
  - (b) Malpractice
  - (c) Battery
  - (d) Assault
95. A client has just been diagnosed with cancer but responded as though this is an impossibility which of the defense mechanisms is being demonstrated here?
- (a) Projection
  - (b) Denial
  - (c) Displacement
  - (d) Repression.
96. Which of the following is not an education tool required prior to an endoscopic procedure?
- (a) the purpose of the procedure
  - (b) what to expect during the procedure
  - (c) how long the procedure will take
  - (d) preparation required prior to the surgery
97. An occult blood examination was ordered for peptic ulcer disease. The specific specimen needed is:
- (a) Stool
  - (b) Blood
  - (c) Sputum
  - (d) Gastric juice
98. A clinical manifestation indicative of ketoacidosis is
- 
- (a) Cold Sweat
  - (b) Kussmaul breathing
  - (c) Hyper reflexia
  - (d) Oedema
99. Addison's disease refers to \_\_\_\_\_
- (a) Pituitary insufficiency
  - (b) Adreno cortical Insufficiency
  - (c) Thyroid Insufficiency
  - (d) Oversecretion of adrenocortical hormones
100. Which of the following is NOT a function of the placenta?
- (a) Waste elimination
  - (b) Respiratory gas transfer
  - (c) Urine formation
  - (d) Hormone production



## SECTION B

1. Mr. Jones a 41years old school teacher reported at the surgical ward outpatient department with an history of difficulty in passing urine of three days duration. On physical examination, the urinary bladder was distended and painful on palpation and temperature was 39 degree centigrade. A medical diagnosis of urine retention Post gonococcal urethral structure was made and Bouginage was scheduled.

a) With the aid of a well labelled diagram, describe the anatomy of the male urethra.

Diagram-1marks

labelling -1 marks Description -3 marks

b) List six (6) nursing measure to relief acute retention of urine (3 marks)

c) Discuss the pathophysiology of post Gonococcal urethra structure (3 marks)

d) Utilizing the nursing care plan, identify and solve in order of priority three(3) nursing diagnosis/problems of Mr. Jones (9 marks)

2. Madam Agnes a 55years old trader who visited the doctor's clinic was diagnosed as having OSTEOARYTHRITIS

a) State four (4) differences osteoarthritis and Rheumatoid arthritis (4 marks)

b) Describe the pathophysiology of acute osteoarthritis ( 5 marks)

c) Using the nursing care plan, identify and solve three (3) nursing diagnosis of Madam Agnes in the first 48hours of admission (9 marks)

d) List four (4) major complications that may arise (2 marks)

3. Mr. Godwin a 38years old civil servant was brought to the outpatient department of your clinic yesterday with multiple open wound all over his body due to bomb explosion in his environment recently

a) Define disaster (2 marks)

b) With four (4) examples in each, identify and explain type of disaster (4 marks)



- c) Describe the management of disaster victims (10 marks)
- d) What the roles of government and community in disaster preparedness (4 marks) .

4. (a) Master Alex, a ten year old primary 5 pupil is admitted into the paediatric

ward with a diagnosis of Lassa fever.

- i. Identify the natural reservoir of its virus. (1 mark)
  - ii. Enumerate six (6) clinical manifestations of Lassa fever. (3 marks)
  - iii. Explain three (3) modes of transmission of the infection. (3 marks)
  - iv. Briefly discuss four (4) measures that may be taken to prevent spread of the infection. (4 marks each)
  - v. List the two (2) complications of Lassa fever. (1 mark)
- (b) Describe four (4) methods of waste disposal in the community. (8 marks)

5. International Council of Nursing (1983) shifted nursing focus from curative care, medically circumscribed and hospital-based orientation to getting involved in all aspects of decision making. Nursing has since been steadily evolving as a “profession sui generis”.

- (a) Identify and discuss the four (4) characteristics of a profession and apply them to Nursing. (8 marks)
- (b) Briefly explain the following:
  - i. Organizing ii. Directing iii. Evaluation (4 marks each)

6. (a) i. Explain the term “Hysteria”. (2 marks)
- ii. List six (6) clinical manifestations of hysteria. (3 marks)
  - iii. Mention two (2) types of hysteria. (1 mark)
  - iv. List four (4) psychological methods used in the management of hysteric patients. (2 marks)
- (b) Write short notes on the following:
- |                                 |                       |
|---------------------------------|-----------------------|
| i. Testamentary Capacity        | ii. Power of attorney |
| iii. Systematic desensitization | iv. Senile Dementia   |
- (3 marks each)



### OBJECTIVE ANSWER

|       |       |
|-------|-------|
| 1. B  | 51. C |
| 2. C  | 52. D |
| 3. B  | 53. C |
| 4. C  | 54. B |
| 5. D  | 55. C |
| 6. A  | 56. C |
| 7. B  | 57. A |
| 8. A  | 58. D |
| 9. B  | 59. A |
| 10. A | 60. A |
| 11. D | 61. D |
| 12. A | 62. B |
| 13. B | 63. C |
| 14. D | 64. D |
| 15. A | 65. B |
| 16. B | 66. D |
| 17. D | 67. B |
| 18. B | 68. D |
| 19. A | 69. A |
| 20. C | 70. B |
| 21. B | 71. C |
| 22. B | 72. C |
| 23. D | 73. C |
| 24. A | 74. D |
| 25. B | 75. A |
| 26. A | 76. A |
| 27. C | 77. D |
| 28. A | 78. A |
| 29. C | 79. C |
| 30. C | 80. C |
| 31. D | 81. B |
| 32. A | 82. D |
| 33. D | 83. B |



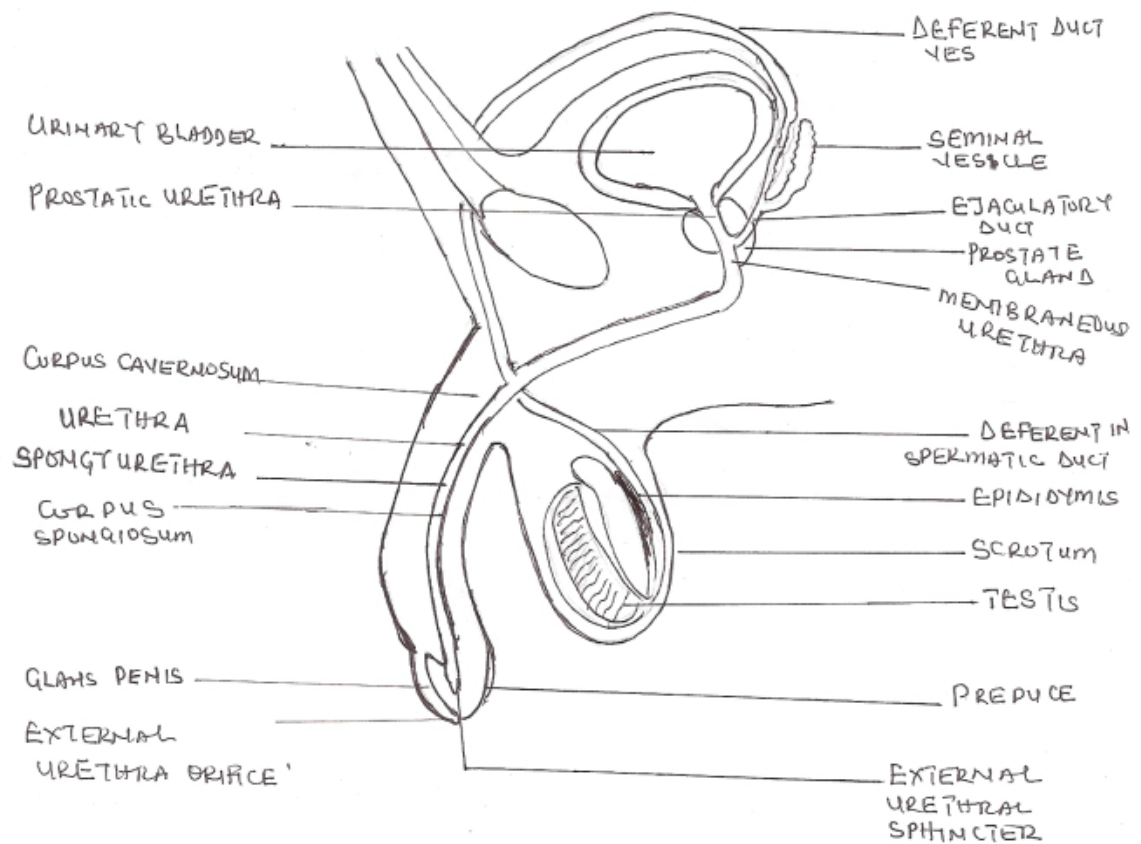
|       |        |
|-------|--------|
| 34. A | 84. D  |
| 35. C | 85. A  |
| 36. A | 86. A  |
| 37. A | 87. C  |
| 38. C | 88. A  |
| 39. D | 89. B  |
| 40. B | 90. B  |
| 41. B | 91. B  |
| 42. D | 92. D  |
| 43. A | 93. C  |
| 44. B | 94. B  |
| 45. D | 95. B  |
| 46. D | 96. D  |
| 47. B | 97. A  |
| 48. B | 98. B  |
| 49. A | 99. D  |
| 50. D | 100. C |

## ANSWER TO SECTION B

### 1a. Anatomy of The Male Urethra



## STRUCTURE OF MALE REPRODUCTIVE ORGAN



The male urethra provides a common pathway for the flow of urine and semen. It is about 19-20cm long and consists of three parts.

-**The prostatic urethra** originates at the urethra orifice of the bladder and passes through the prostate gland.

-**The membranous urethra** : is the shortest and narrowed part and extends from the prostate gland to the bulb of the penis after passing through the perineal membrane.

-**The spongiose or penile urethra** lie within the corpus spongiosum of the penis and terminates at the external urethra orifice in the gland penis.

There are two urethral sphincter

-The internal sphincter consists of smooth muscle fibres at the neck of the bladder above the prostate gland.

-The external sphincter consists of skeletal muscle fibres surrounding the membrane part

The male Urethra and related organs

The following labels are important

i) Internal urethra sphincter, membranous sphincter

ii) Ejaculation duct, spongy urethra

iii) Parts of urethra

iv) External urethra sphincter

Diagram 1mark

Labelling mark

Description 3marks

### 1b) NURSING MEASURES TO RELIEVE ACUTE RETENTION OF URINE

-Providing privacy

-Ensure an environment and position conducive to voiding

-Assisting the patient to use the bathroom or commode rather than bed pan

-Application of warm compress over the pubic area

-Encouragement and reassurance

-Turning on the tap nearby why patient is trying to void

-Stroking the abdomen or inner aspect of the thigh as well as the suprapubic area

-Dipping the patient's hand in warm water.

-Catheterization.

( ½ mark for any 6 points=3 marks)

### 1c) Pathophysiology of Gonococcal Urethral Stricture

-The urethral stricture may occur due to scar tissue formation resulting from gonococcal infection (inflammation and healing by scarring).

-The fibrotic tissue narrows the urethral passage or lumen. The presence of the stricture prevents easy passage of urine from the urinary bladder through the urethra leading to urinary retention in the bladder.

-In complete retention, there will be frequent passage of small amount of urine which will be painful and dribbling of urine occurs





-There will be passage of purulent urethral discharge with some itching and burning sensations of the urethral meatus.

-The retained urine in the bladder contains a lot of toxic waste which can infect the bladder causing cystitis. With no prompt intervention, the full bladder causes backflow of urine through the ureters (hydroureter) then to the pelves of the kidney causing hydronephrosis which may lead to pyelonephritis and subsequently renal failure. (3 marks).

1d. THE NURSING CARE PLAN OF MR. JONES, A 41 YEAR OLD SCHOOL TEACHER WITH THE

#### DIAGNOSIS OF URETHRA STRICTURE

| S/ No | Nursing Diagnosis                                                               | Nursing Objectives                                                                                     | Nursing Order                                                                                                                                                           | Scientific Rationale                                                                                                                                                                                                                                             | Evaluation                                                                        |
|-------|---------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------|-------------------------------------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------|
| 1     | Altered Urinary elimination pattern related to urethral stricture<br><br>1 mark | Patient will empty bladder (pass urine) with ease within 20-40 minutes of nursing action<br><br>1 mark | 1.Open a nearby tap<br><br>2.Apply warm compress on the pubic area<br><br>3.Obtain an order for catheterization<br><br>4. Monitor intake and output chart<br><br>1 mark | 1 & 2 will stimulate a patient to void urine<br><br>This allows the urine to pass catheter (urinary) thereby relieving urine retention.<br><br>To help maintain the balance between fluid intake and output thereby preventing electrolyte imbalance<br>1 ½ mark | Patient emptied his bladder after 20 minutes of nursing intervention<br><br>1mark |
| 2.    | Pain related to bladder distension and difficulty in nicturition                | Patient will empty bladder with ease within 30minutes – 1 hour of nursing action                       | -Put patient in a conducive position<br>-Encourage patient to void by opening nearby tap<br>-Reassure patient<br>-Give prescribed analgesics                            | -Relaxes muscle and reduces pain<br>-Voiding relieves pain<br>-Relieves pain psychologically<br>-Warm compress increases blood supply and relieves                                                                                                               | Patient empties bladder with ease after 40 minutes of nursing action.             |



|    |                                               |                                                                                        |                                                                                                                                              |                                                                                                                                                                                                                                          |                                                                                                                                         |
|----|-----------------------------------------------|----------------------------------------------------------------------------------------|----------------------------------------------------------------------------------------------------------------------------------------------|------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------------|-----------------------------------------------------------------------------------------------------------------------------------------|
|    |                                               |                                                                                        | -Assist in suprapubic tap                                                                                                                    | pain<br>-Analgesics act on pain perception<br>-This will relieve bladder retention thereby relieving pain                                                                                                                                |                                                                                                                                         |
| 3. | Altered thermoregulation related to infection | Patient's temperature will reduce by 1°C within 30 minutes to 1 hour of nursing action | -Expose patient<br>-Open nearby window and put on fan<br>-Tepidsponge patient<br>-Give prescribed antipyretic and antibiotics                | -Exposure reduces temperature by convection (for 1 & 2)<br>-Reduces temperature by conduction and evaporation<br>-Acts on the temperature regulating centre in the medulla oblongata<br><br>-Anti biotics will help to combat infection  | Patient's temperature was reduced by 1°C after 40 minutes of nursing action                                                             |
| 4. | Anxiety related to outcome of surgery         | Patient will be calm within 12-25 hours of hispitalization                             | 1.Establish rapport<br>2.Provide privacy<br>3.Allow patient to verbalise<br>4.Explain need for surgery<br>5.Give prescribed anxiolytic agent | 1.Makes patient feel accepted<br>2.Helps patient to open up without restriction<br>3.Verbalisation helps to relive fear.<br>4.Explanation will help ease acceptance of surgery<br>5.Anxiolytic agent depress the CNS and induce hypnosis | Patient was calm within 20 hours of hospitalization.<br><br>3 marks for each correct identified and solved prob/diagnosis<br><br>9marks |

